Saathi clinical review

Version 2 · 3 November 2026 · For Dr. Krithika S Rao · Prepared by Tushar Agarwal · Synthetic demo, not for real patients

> **How to review:** for each item, tick **Approve**, **Change** or **Remove** and write any correction underneath. You can annotate this PDF, or reply on WhatsApp with the item number (for example "B1 change: …"). Nothing here is shown to families or treated as approved until you confirm it.

A. Decisions from our call on 3 November: please confirm

I wrote these down during our call. Tick Approve if I understood correctly.

A1. What Saathi is

An extended, out-of-hours support and continuity service. It triages caregiver concerns into a teleconsultation call-back; it does not give treatment by chat.

☐ Approve ☐ Change

A2. Check in to Saathi from the hospital record

At the end of an OPD visit, the doctor clicks "Check in to Saathi" in the hospital system. Saathi receives the patient's details, prescription, notes and follow-up plan (for example a phone follow-up in 2 weeks and an OPD visit in 1 month). For the demo, a dummy hospital page will hold one synthetic case that you write.

☐ Approve ☐ Change

A3. Reminders on WhatsApp

Morning and evening medicine reminders listed by medicine, refill reminders, OPD appointment reminders and phone follow-up reminders. Families tap "Given" or "Not given", which builds the medicine-compliance chart.

☐ Approve ☐ Change

A4. What "Acknowledge" means

The on-call doctor acknowledging a report means it is accepted into the call-back list. It does not close the report. The report closes only after the call, when the doctor writes call notes and the advice given.

☐ Approve ☐ Change

A5. Call-back targets

Urgency	Who	Call back within
RED	On-call doctor (junior doctor / PG after 5 pm)	1 hour
AMBER	Nurse in charge assigns doctor, nurse or psychologist	6 hours (you mentioned 6 or 8)
GREEN	Next working day	24 hours

Please write the exact numbers you want:

☐ Approve ☐ Change

A6. Three kinds of login

(1) Doctors, nurses and psychologists see full medical details. (2) Non-medical staff (call manager) see only date-wise counts, urgency, status, patient name and hospital ID, and can export a daily PDF report. (3) Patients and families sign in with a one-time code by phone number or email.

☐ Approve ☐ Change

A7. Fees (optional per organisation)

Paid services can ask for a call-back fee when the family requests a call (audio or video), with follow-up pricing (for example the first follow-up within 15 days at half fee). Free services such as NGOs switch fees off. The admin sees a "fee paid" checklist. No payment is collected in the demo.

☐ Approve ☐ Change

A8. Prescriptions after a teleconsult

After the call the doctor may write a prescription with a digital signature, sent to the family on WhatsApp. Following the Telemedicine Practice Guidelines, Saathi blocks Schedule X, narcotic and psychotropic drugs and anti-cancer drugs on teleconsult prescriptions. Records are kept for 3 years under the DPDP Act and IT rules.

☐ Approve ☐ Change

A9. Demo language and channel

English only for the demo. WhatsApp and a web page for families. Telegram is not needed.

☐ Approve ☐ Change

B. Messages families would receive

B1. Message on any RED report (your wording from the call)

> The patient's condition may be critical and cannot be managed through teleconsultation. Please seek urgent medical care at the nearest hospital now. If breathing stops or you cannot wake the patient, call 108. Our on-call doctor has also been alerted.

Should this go for every RED rule, or only some (for example R3 breathing and R1/R2 drowsiness)? Should "caregiver not coping" (R13) get a different message?

☐ Approve ☐ Change ☐ Remove

B2. Message on AMBER or GREEN (drafted by Tushar)

> Thank you. Your concern has been recorded and the care team will call you within [6 hours / 24 hours]. If the patient becomes very sick, please go to the nearest hospital or call 108.

☐ Approve ☐ Change ☐ Remove

B3. Rider shown at registration and in every reply (drafted; legal review pending)

> Saathi is a support service for patients already under our care. It is not an emergency service. If the patient is very sick, go to the nearest hospital or call 108 immediately.

☐ Approve ☐ Change

B4. Emergency card (older draft; may now be replaced by B1)

> 1. Your care team has been alerted. A doctor will call you within [N] minutes. You can also call the on-call doctor now: [on-call number].
> 2. Do not give another extra dose of pain medicine until the doctor speaks to you.
> 3. Stay with the patient. If they are lying down, turn them gently onto their side.
> 4. If breathing becomes very slow or stops, or you cannot wake them, call 108 now.
> 5. Keep the medicine box and this phone with you.

☐ Approve ☐ Change ☐ Remove

B5. C6 Constipation while taking morphine

1. Morphine almost always causes constipation.
2. Give the laxative every day, even if the patient has passed stool.
3. Offer fluids often.
4. Gentle movement helps.
5. Tell the team if there is no stool for 3 days, or if there is belly pain or vomiting.

☐ Approve ☐ Change ☐ Remove

B6. C12 Counting breaths and checking drowsiness

Counting breaths: wait until the patient is resting quietly. Watch or feel the chest or tummy rise. Count each rise for one full minute by the phone timer.
• 12 or fewer → RED: the doctor is alerted.
• 13–15 → the team reviews today (pending your decision, see D2).
Checking drowsiness: call their name and talk to them. If they keep falling asleep while talking or eating, or are hard to wake, the doctor is alerted immediately.

☐ Approve ☐ Change ☐ Remove

B7. C2 Giving opioid medicines and applying a fentanyl patch

The full draft is long (liquid morphine steps, nine fentanyl patch steps). It is in the clinical review desk of the app; I can send it separately. Should families receive this card at all, given the guidance not to advise on medicines by WhatsApp?

☐ Keep ☐ Change ☐ Remove

C. Triage rules: when is a report RED or AMBER?

Saathi applies these rules exactly as written; the AI never decides urgency. "Drowsiness" levels: W1 awake and talking · W2 sleeping, wakes easily · W3 a little drowsy, stays awake · W4 keeps falling asleep while talking or eating · W5 very hard to wake or cannot be woken.

Rule	Urgency	Becomes this urgency when the family reports…	Your decision	Comments
R1	**RED**	Patient is very hard to wake or cannot be woken (W5).	□ Approve □ Change □ Remove	
R2	**RED**	Patient keeps falling asleep while talking or eating (W4).	□ Approve □ Change □ Remove	

Rule	Urgency	Becomes this urgency when the family reports…	Your decision	Comments
R3	**RED**	Any of: breaths counted over a full minute are 12 or fewer; breathing is very slow or has pauses; or new noisy or rattling breathing while the patient is drowsy (W3–W5).	□ Approve □ Change □ Remove	
R4	**RED**	Any of: sudden severe breathlessness at rest; blue lips; or face or neck swelling with breathlessness.	□ Approve □ Change □ Remove	
R5	**RED**	New back pain with leg weakness, or new loss of bladder or bowel control.	□ Approve □ Change □ Remove	
R6	**RED**	Sudden confusion, or seeing or hearing things that are not there.	□ Approve □ Change □ Remove	
R7	**RED**	A seizure (fit).	□ Approve □ Change □ Remove	
R8	**RED**	Any of: heavy bleeding; vomiting blood or coffee-ground material; black stools.	□ Approve □ Change □ Remove	
R9	**RED**	Vomiting 2 or more times in 24 hours, with a swollen belly and no stool or wind. **Is "repeated" 2 or more?**	□ Approve □ Change □ Remove	
R10	**RED**	Temperature 38 °C or more, with chemotherapy in the last 6 weeks.	□ Approve □ Change □ Remove	
R11	**RED**	Sudden pain of 8/10 or more, not helped by the rescue dose.	□ Approve □ Change □ Remove	
R12	**RED**	A medicine mistake (wrong dose, wrong medicine or double dose).	□ Approve □ Change □ Remove	
R13	**RED**	Thoughts of self-harm or a crisis, or the caregiver says they are not coping at all.	□ Approve □ Change □ Remove	
A1	**AMBER**	3 or more rescue doses in 24 hours (your confirmed rule).	□ Approve □ Change □ Remove	
A2	**AMBER**	Pain 7/10 or more despite medicines, or pain 3 or more points above the patient's usual score.	□ Approve □ Change □ Remove	

Rule	Urgency	Becomes this urgency when the family reports…	Your decision	Comments
A3	**AMBER**	Vomiting 3 or more times in 24 hours, or unable to keep medicines down.	□ Approve □ Change □ Remove	
A4	**AMBER**	No stool for 3 days while on morphine.	□ Approve □ Change □ Remove	
A5	**AMBER**	Only sips or mouthfuls, or nothing, for 24 hours or more.	□ Approve □ Change □ Remove	
A6	**AMBER**	Newly a little drowsy (W3) within 72 hours of a dose increase.	□ Approve □ Change □ Remove	
A7	**AMBER**	New jerking movements.	□ Approve □ Change □ Remove	
A8	**AMBER**	Confusion that has come on gradually.	□ Approve □ Change □ Remove	
A9	**AMBER**	Wound changes (more bleeding, stronger smell, new discharge or spreading redness), when a wound photo is available. **Should a missing photo matter?**	□ Approve □ Change □ Remove	
A10	**AMBER**	New breathlessness at rest.	□ Approve □ Change □ Remove	
A11	**AMBER**	2 or more regular doses missed, or morphine supply for 5 days or fewer.	□ Approve □ Change □ Remove	
A12	**AMBER**	Caregiver says they are struggling on 2 check-ins in a row.	□ Approve □ Change □ Remove	
A13	**AMBER**	A family question is still unanswered after 2 care-card attempts.	□ Approve □ Change □ Remove	
A14	**AMBER**	No check-in for 2 days.	□ Approve □ Change □ Remove	
A15	**AMBER**	Breaths counted over a full minute are 13 to 15. **Currently switched off, see D2**	□ Switch on □ Change □ Remove	

D. Questions only you can answer

D1. Is anything missing?

Are there palliative emergencies that should be RED or AMBER but are not listed above (for example uncontrolled agitation, inability to swallow, a fall)?

D2. 13–15 breaths per minute in adults: AMBER?

☐ Yes, AMBER ☐ No ☐ Other

D3. Safety test results (60 invented messages in English, Hindi and Kannada)

Saathi caught 27 of 30 expected REDs. It missed "seeing people who are not there" in Hindi and "I am not coping at all" in Hindi and Kannada. Could you check the test labels and add about 20 cases from your experience, with details changed?

☐ Yes ☐ Later

D4. The dummy hospital case

Please write one synthetic case: name, age, address, diagnosis, current prescription (medicine, dose, morning/evening timing, rescue dose), notes and follow-up plan.

D5. Videos

Which of the shortlisted videos should families receive? (Karunashraya Kannada, NHSRC Hindi, hospital English videos for fentanyl patch, liquid morphine, counting breaths, constipation, mouth care, pressure care.)

D6. Children's pack

Adult rules are never applied to children. Should the demo leave children out entirely?

☐ Leave out for demo ☐ Include